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The quest to find genuine adult ADHD: Navigating diagnostic challenges of malingering - a review

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ABSTRACT

Background: Attention-Deficit/Hyperactivity Disorder (ADHD) is a neurodevelopmental disorder characterized by persistent patterns of inattention, hyperactivity, and impulsivity that impair daily functioning. Accurate diagnosis is therefore essential, particularly in adulthood, where symptom reporting may be influenced by external incentives. Malingering, defined as the deliberate fabrication or exaggeration of symptoms for external gain, is a relevant concern in ADHD assessment, particularly for academic accommodations, stimulant medications, and avoidance of responsibilities. This review examined current evidence on the detection of malingered presentations in adult ADHD assessment.

Methods: We searched PubMed, Scopus, and Web of Science for empirical studies examining feigned, malingered, exaggerated, or otherwise noncredible ADHD presentations and 29 peer-reviewed studies met the inclusion criteria.

Results: Detecting malingering in ADHD remains challenging. Validity measures showed heterogeneous effectiveness, and ADHD symptom scales appeared vulnerable to manipulation. However, the available evidence suggests that performance validity tests, including the Test of Memory Malingering (TOMM), Word Memory Test (WMT), Letter Memory Test (LMT), Digit Memory Test (DMT), and Nonverbal Medical Symptom Validity Test (NV-MSVT), may help differentiate genuine ADHD from noncredible presentations when interpreted within a multi-method assessment framework.

Limitations: Most studies relied heavily on self-report symptom measures, used heterogeneous validity criteria, and lacked independent external corroboration, limiting comparability across findings. Evidence largely comes from the United States, and longitudinal data remain scarce.

Discussion: Distinguishing ADHD from malingering requires a specialist-led assessment. Greater standardization of diagnostic protocols and validity-assessment procedures remains urgently needed.

1. Introduction

1.1. Breaking down ADHD into pieces: beyond DSM-5 definitions

Attention-Deficit/Hyperactivity Disorder (ADHD) is defined in the DSM-5-TR as a persistent pattern of inattention and/or hyperactivity-impulsivity that interferes with functioning and/or development (American Psychiatric Association, 2022). For a diagnosis to be established, symptoms must have been present before the age of 12 and must be observed in two or more settings, such as home and school or work, to establish that the disorder affects the individual across different environments (American Psychiatric Association, 2022). In addition, the symptoms must not be better explained by other medical or psychiatric conditions, such as mood, anxiety, dissociative, or personality disorders, and must not occur exclusively during the course of schizophrenia or another psychotic disorder (American Psychiatric Association, 2022; Drechsler et al., 2020). For children and adolescents, a person must exhibit at least six symptoms of inattention and/or hyperactivity-impulsivity for at least six months, whereas only five symptoms are required for individuals aged 17 years and older.

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Symptoms of inattention include difficulty paying close attention to details, making careless mistakes, difficulty sustaining attention, not listening when spoken to directly, failing to follow through on instructions, and avoiding tasks that require sustained mental effort (American Psychiatric Association, 2022).

Symptoms of hyperactivity and impulsivity include fidgeting, leaving one's seat when remaining seated is expected, subjective or observable restlessness, difficulty engaging quietly in activities, excessive talking, interrupting others, difficulty waiting one's turn, and acting before questions or instructions are completed (American Psychiatric Association, 2022).

Although the DSM-5-TR criteria include specifiers for presentation and severity, these classifiers may fail to capture the fluctuating nature of ADHD, in which symptom severity and presentation can vary over time (Faraone et al., 2024). In addition, comorbid psychiatric conditions may often obscure the clinical picture, making differential diagnosis particularly challenging (Drechsler et al., 2020).

Mood disorders, for example, can cause attentional and concentration difficulties, as well as fatigue and irritability, all of which are also common in ADHD (Asherson et al., 2014). Similarly, early obsessive-compulsive disorder (OCD) symptomatology can resemble ADHD, as intrusive and ruminative thoughts may lead to restlessness and concentration difficulties, thereby overlapping with core ADHD symptoms (Abramovitch et al., 2015).

Similar attentional difficulties might also be explained by personality disorders, which may modify their presentation across the lifespan and should be considered in explaining the symptoms (Weibel et al., 2020).

Affective temperaments, particularly cyclothymic temperament, can generate fluctuations in energy, attention, and goal directed behavior that resemble ADHD, yet temperaments are typically conceptualized as relatively stable traits rather than a neurodevelopmental syndrome (Syrstad et al., 2020; Kopala-Sibley et al., 2018).

Finally, as well described in the last decades, gender differences further complicate the issue, as ADHD is diagnosed more frequently in boys during childhood, whereas this difference appears to narrow in adulthood (Quinn and Madhoo, 2014; Young et al., 2020).

Importantly, ADHD does not always persist into adulthood to the extent previously assumed. Prospective studies that followed children diagnosed with ADHD into adulthood found that a substantial proportion no longer meet full diagnostic criteria as adults (Sibley et al., 2017; Caye et al., 2016).

In addition, adult ADHD evaluations currently too often rely heavily on self-reported symptoms, particularly when childhood records or collateral informants are unavailable (Abdelnour et al., 2022). The increasing availability of online self-assessment tools and mental health content on social media may further amplify concerns about overdiagnosis and incorrect self-diagnosis (Karasavva et al., 2025).

Taken together, these considerations indicate that adult ADHD assessment should not depend on subjective symptom endorsement alone. Without adequate professional oversight and corroboration, individuals may respond in ways that confirm preconceived beliefs or may shape their responses to meet diagnostic criteria, thus undermining diagnostic validity (Brevik et al., 2020).

1.2. Feigning versus malingering: definitions and clinical relevance

In the context of adult ADHD assessment, it is essential to distinguish between feigning and malingering. Both may contribute to misdiagnosis, but they are not equivalent. Feigning refers broadly to the simulation, fabrication, or exaggeration of symptoms, regardless of whether a clear external incentive is documented, whereas malingering is narrower: it refers to the intentional production or exaggeration of symptoms motivated by external incentives (American Psychiatric Association, 2022; McCullumsmith and Ford, 2011). This distinction is particularly relevant in ADHD, where many symptoms are subjective and therefore potentially vulnerable to misinterpretation or intentional distortion (Edmundson et al., 2017).

Feigning may also occur in the context of psychological distress, diagnostic uncertainty, or other psychiatric conditions and may not always be fully conscious. For example, a student who feels overwhelmed by academic pressure may report concentration difficulties not to obtain a formal advantage, but as a way of expressing distress or impairment that is subjectively experienced as real. This presentation should not be equated with malingering unless there is evidence of intentional distortion motivated by external gain (Bush et al., 2005).

Malingering, by contrast, involves a conscious and intentional attempt to obtain a benefit. In the case of ADHD, this may include seeking stimulant medications, obtaining academic accommodations, or avoiding occupational, academic, or legal responsibilities (Sansone and Sansone, 2011).

Symptom validity tests (SVTs) assess the credibility of reported symptoms and are primarily used to detect over-reporting, atypical symptom endorsement, or implausible self-presentations (Larrabee, 2012; Sweet et al., 2021). Performance validity tests (PVTs) assess the credibility of cognitive test performance and are primarily used to detect noncredible cognitive underperformance (Bush et al., 2005; Sweet et al., 2021). This distinction is important because SVTs and PVTs measure related but different response-validity constructs (Larrabee, 2012; Ord et al., 2021b). In adult ADHD assessment, SVTs may help evaluate exaggerated symptom endorsement, whereas PVTs may help determine whether cognitive performance is interpretable before drawing conclusions about attention, working memory, or processing speed (Marshall et al., 2016; Ovsiew et al., 2023).

This distinction is especially important because the diagnosis of ADHD in adulthood relies to a large extent on the patient's account during the clinical interview. When symptom reporting is not carefully contextualized and corroborated, the diagnostic process becomes particularly vulnerable to noncredible presentations. Among the most commonly described motivations for malingering ADHD are academic benefits, such as extended examination time or other accommodations, access to stimulant medications such as methylphenidate or amphetamine, and the avoidance of responsibilities or the justification of poor performance in academic, occupational, or other settings (Harrison and Rosenblum, 2010; Arria et al., 2008; Fuermaier et al., 2021).

For this reason, ADHD assessment should not rely exclusively on self-report. In-depth interviews, behavioral observations, collateral information from family members, teachers, or other informants, ADHD symptom measures, and explicit validity assessment may all help determine whether reported symptoms are consistent with the overall clinical presentation (Srinath et al., 2019). This approach is also important because feigned or exaggerated ADHD-like symptoms may arise in the context of other psychiatric conditions, as discussed above. In cases of malingering, the consequences may extend beyond diagnostic error. Inappropriate access to stimulant medication may expose individuals to adverse effects, including insomnia, appetite suppression, anxiety, cardiovascular complications, and misuse or diversion, in addition to potential legal consequences when medical resources are obtained fraudulently (McCullumsmith and Ford, 2011). These considerations further support the need to clarify which methods are most useful for identifying malingering and noncredible presentations in adult ADHD assessment.

2. Methodology

2.1. Analysis of the literature review

A comprehensive literature search was conducted to identify empirical studies examining malingering or feigning of attention-deficit/hyperactivity disorder (ADHD) in adult or transition-age assessment contexts. The search was performed in PubMed, Scopus, and Web of Science using the Boolean query: ((malingering OR feigning) AND (ADHD OR “attention deficit hyperactivity disorder”)). The search

strategy was intentionally restricted to the terms malingering and feigning because the review specifically focused on the detection of intentionally fabricated or exaggerated ADHD symptoms in adult diagnostic settings, particularly when such presentations may be associated with external incentives. Broader or conceptually distinct terms, including constructs related to factitious presentations, were not included because they reflect different motivational frameworks and are used less consistently in the adult ADHD literature, which would have reduced the specificity of the search. Records published up to December 13, 2024, were considered. Eligible studies were required to present original empirical data, focus on malingering in adult ADHD, provide full-text availability in English or in a version allowing reliable translation, and report a clear methodological framework for assessing malingering. Studies exclusively addressing pediatric populations under 18 years of age, non-original research formats such as reviews, editorials, or abstracts only, investigations not primarily focused on malingering in ADHD, and non-peer-reviewed or inaccessible articles were excluded during the study selection process. Accordingly, studies were retained only when they empirically manipulated, classified, measured, or validated feigned, malingered, exaggerated, or otherwise noncredible ADHD presentation in an adult or transition-age ADHD assessment context.

A total of 305 records were initially identified. After the removal of 163 duplicates, 142 records remained for title and abstract screening. At this stage, 72 records were excluded because they did not meet the inclusion criteria, and 70 reports were sought for full-text retrieval. Of these, 7 could not be obtained despite attempts through major online repositories, leaving 63 articles for full-text assessment. Following detailed evaluation, 34 articles were excluded: 11 because they were non-original research, 2 because they focused exclusively on pediatric

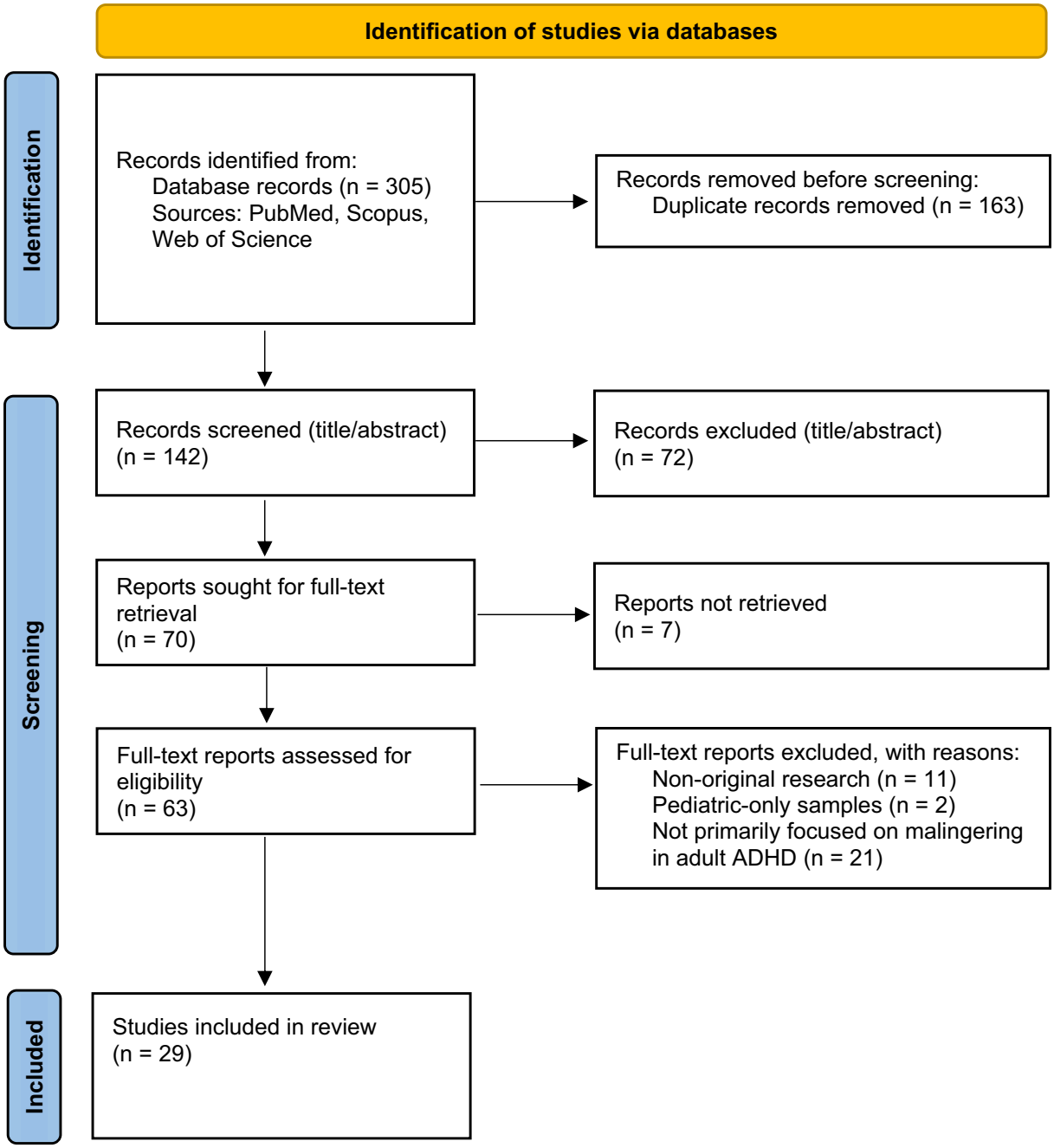


Fig. 1. PRISMA 2020 flow diagram of the study selection process, adapted from Page et al. (2021).

populations, and 21 because they did not primarily address malingering in ADHD. Ultimately, 29 studies met all inclusion criteria and were included in this review. The final set of articles provides a comprehensive basis for analyzing methodologies and findings relevant to the detection and characterization of malingering in adult ADHD. Fig. 1 presents the PRISMA flow diagram of the study selection process.

3. Results

Table 1 summarizes the characteristics of the 29 studies included in the review.

3.1. Validity tests and indices used to detect malingering

Most of the reviewed studies used a range of ADHD symptom measures and methodological approaches to detect malingering or other forms of noncredible performance. Across the 29 included studies, TOMM was used in 6 studies (20.7%), WMT in 5 studies (17.2%), LMT in 4 studies (13.8%), DMT in 4 studies (13.8%), and NV-MSVT in 4 studies (13.8%). When MSVT and NV-MSVT were considered together as related memory-based PVTs, they appeared in 5 studies (17.2%). PAI or PAI-A validity indicators were examined as focal validity measures in 5 studies (17.2%). Reported diagnostic performance varied substantially by instrument, cut-off, sample composition, and study design. Among studies reporting explicit classification values, sensitivity estimates ranged from 47.5% to 94%, whereas specificity estimates ranged from 80.1% to 100%. For example, Jasinski et al. (2011) reported that failure of two or more validity tests produced sensitivity of 0.475 and specificity of 1.00. Aita et al. (2018) reported sensitivity of 54% and specificity of 92% for a scale-based PAI algorithm and sensitivity of 85% and specificity of 97% for an item-level PAI algorithm. Ramachandran et al. (2019) reported sensitivity of 90.3% and specificity of 80.1% for the Subtle ADHD Malingering Screener (SAMS), whereas Quinn (2003) reported sensitivity of 94% and specificity of 91% for the IVA CPT impairment index. A WMT-specific sensitivity range could not be summarized consistently because WMT was often used as part of a broader validity battery, as a criterion measure, or as a cross-check rather than as an isolated index with separately reported classification accuracy. Nevertheless, WMT-related findings were clinically relevant: Suhr et al. (2008) reported a WMT failure rate of 31% in adults referred for ADHD evaluation, and Marshall et al. (2016) included WMT within validity-based criteria used to classify suspect effort in archival adult ADHD assessments. Thus, for WMT, the review could quantify frequency of use but not derive a valid instrument-specific sensitivity range from the available studies without mixing different analytic roles, including stand-alone PVT use, criterion classification, and inclusion within broader validity batteries.

Among stand-alone PVTs, TOMM and WMT were the most frequently represented measures, although frequency of use should not be interpreted as evidence of definitive superiority. These memory-based PVTs include tasks expected to be performed adequately by most individuals with genuine cognitive difficulties. Markedly poor performance on such tasks may therefore suggest noncredible cognitive performance rather than true ADHD-related impairment, although PVT failure should not automatically be equated with malingering. The TOMM, for example, assesses visual recognition memory and was among the most frequently used stand-alone performance validity measures across studies. The WMT similarly compares performance across easier and more difficult memory conditions to detect improbable response patterns. Spenceley et al. (2022) also highlighted the potential value of embedded validity indicators derived from the Woodcock-Johnson IV Tests of Cognitive Abilities, suggesting that such measures may provide useful adjunctive information within a broader validity-oriented assessment framework.

Other measures included the Medical Symptom Validity Test (MSVT), the Nonverbal Medical Symptom Validity Test (NV-MSVT), and the Amsterdam Short Term Memory Test (AKGT), along with psychological validity indicators from the Personality Assessment Inventory (PAI), including Negative Impression Management (NIM), the Malingering Index (MAL) and the Rogers Discriminant Function (RDF), the latter originally developed in forensic settings. Taken together, these findings suggest that performance validity tests and psychological validity indicators may provide an important validity-oriented complement to ADHD symptom scales.

Musso et al. (2016) reported that standard PAI validity cutoffs showed limited sensitivity for detecting feigned ADHD, whereas alternative cutoffs for NIM, MAL, and RDF improved detection while maintaining high specificity. Similarly, Young and Gross (2011) showed that standard ADHD rating scales can be easily manipulated by individuals instructed to simulate symptoms, raising important concerns about the validity of self-reported data in diagnostic settings. Marshall et al. (2016) further observed that the absence of effort testing during initial evaluations may contribute substantially to diagnostic error.

Several studies also support the value of multi-method validity assessment. Williamson et al. (2014) showed that combining TOMM, the Nonverbal Medical Symptom Validity Test (NV-MSVT), and the Letter Memory Test (LMT) improved the differentiation of genuine and simulated ADHD presentations. This approach appears particularly important because no single measure is sufficient on its own. Instead, combining ADHD symptom scales, SVTs, PVTs, collateral developmental information, behavioral observations, and cognitive data may improve interpretive confidence and may reduce the likelihood that current diagnostic protocols are exploited through coached or exaggerated self-presentation.

3.2. ADHD symptom measures and diagnostic information

A wide range of instruments was used to assess ADHD, each with different functions. Among the most frequently used ADHD symptom measures were the Conners' Adult ADHD Rating Scales (CAARS), the Barkley Adult ADHD Rating Scale (BAARS-IV), the Wender Utah Rating Scale (WURS), and related self-report instruments. The Personality Assessment Inventory (PAI) was also used in several studies, not as an ADHD symptom measure, but as a measure of personality, psychopathology and response style. The CAARS assesses core ADHD symptoms such as inattention, hyperactivity, and impulsivity. Smith et al. (2017) reported that participants simulating ADHD scored higher on negative response distortion indices and lower on PIM than genuine ADHD groups, underscoring the vulnerability of self-report scales to noncredible responding. Similarly, Aita et al. (2018) showed that the PAI may contribute useful specificity in the assessment process, although it should not be considered a stand-alone ADHD measure. Other instruments included the Brown Attention-Deficit Disorder Scale (BADDS), used to assess cognitive and organizational difficulties, and the Adult ADHD Self-Report Scale (ASRS), often used for initial screening because of its brevity and ease of administration. Some studies also included continuous performance tests and related computerized measures. Together, these scales show that most studies relied on established instruments to assess ADHD symptomatology. At the same time, several of these measures, particularly self-report scales such as the CAARS and BAARS-IV, are susceptible to malingered responses. Furthermore, accurate diagnosis requires information from multiple sources, not only from the patient, but also, when possible, from parents or other informants, even in adulthood. This helps reduce the impact of subjective distortion and improves developmental reconstruction.

3.3. Geographic distribution of studies

Most of the included studies were conducted in the United States, reflecting the strong emphasis of this literature on North American adult ADHD assessment and college-based evaluations. A substantial proportion of these studies were also conducted in college settings, a pattern that was less common outside the United States. Outside the United

Table 1
Characteristics of the studies included in the review.

Author (year)	Sample, N, Mean Age (SD) or Age Range, Gender %	Design	Country	Comparator	ADHD Measures	Validity measures	Comorbidity	Incentives	Assessment Type	Key Findings	Utility synthesis
Aita et al. (2018)	Clinical ADHD: n = 142, age 21.77 (3.99), 54.9% male. ADHD simulators: n = 138, age 19.83 (1.54), 26.1% male. Mood/ anxiety disorders: n = 137, age 22.71 (4.58), 35.0% male. Clinic no diagnosis: n = 111, age 22.05 (5.07), 57.7% male. Non-treatment-seeking healthy controls: n = 133, age 19.18 (1.57), 30.8% male.	Algorithm development and group classification study using combined archival clinical data and experimental simulation; logistic regression at scale and item level.	USA	Non-treatment-seeking healthy controls and clinic no-diagnosis controls.	Comprehensive psychoeducational/ neuropsychological evaluation (clinical records); PAI (PAI) as focal instrument.	Embedded PAI validity indicators and supplemental indices; effort/ validity screening (including Reliable Digit Span in clinical sample); newly derived PAI adult ADHD malingering indices (scale-based and item-based).	Separate clinical group with mood/ anxiety disorders; exclusion of serious mental illness and intellectual disability (per study criteria).	Academic accommodations and stimulant medication access in college/ university context; broader secondary gain in ADHD evaluations.	Self-report personality inventory with embedded symptom validity indicators (SVT) plus multivariate classification.	Scale-based algorithm: sensitivity 54% and specificity 92% for distinguishing ADHD simulators from genuine ADHD. Item-level algorithm: sensitivity 85% and specificity 97%. New indices outperformed most existing PAI validity indicators across comparison groups.	Supported. PAI-derived, ADHD-specific embedded validity algorithms showed high specificity and potentially useful sensitivity, particularly at the item level, although further validation in real-world clinical referrals remains necessary.
Berger et al. (2021)	Two-phase design. Phase 1 prospective sample: n = 47 per group (simulators, ADHD patients, healthy controls). Mean age: simulators 22.70 (2.24), ADHD 23.79 (2.17), controls 23.15 (2.39). Gender (male/female): simulators 10/37, ADHD 17/30, controls 14/33. Phase 2 retrospective archival comparison: 47 ADHD patients and 47 age-matched healthy controls.	Phase 1 prospective simulation study developing MOXO-d-CPT validity cutoffs, followed by Phase 2 retrospective validation using archival data.	Israel (with archival data sourced across multiple sites in Phase 2)	Healthy controls and clinically diagnosed ADHD patients; simulators served as noncredible comparison group.	MOXO-d-CPT (attention, timeliness, impulsivity, hyperactivity indices); WURS-25 and ASRS used as symptom measures.	Established performance validity test (WMT indices reported) used alongside MOXO-d-CPT; validity classification cross-checked with WMT variables (e.g., Immediate Recognition, Delayed Recognition, Consistency).	Not a central focus; inclusion/ exclusion criteria applied for ADHD diagnosis and control status.	Not manipulated directly; study framed within typical incentives motivating malingered ADHD presentations (accommodations and medication access).	Computerized CPT with embedded indices, combined with performance validity testing, and multivariate classification (logistic regression and combined scale).	Simulators performed significantly worse on all MOXO-d-CPT indices than healthy controls and ADHD patients. A combined MOXO-d-CPT scale based on attention, hyperactivity, and impulsivity showed strong discrimination and good classification accuracy in both prospective and archival comparisons.	Supported. MOXO-d-CPT indices, especially when combined into a composite score, showed promising discrimination of malingered ADHD, although replication in independent clinical samples is still needed.
Bryant et al. (2018)	College students: N = 167. ADHD group n = 36, age 19.69 (6.36), 66% male. Best-effort controls n = 44, age 19.25 (3.90), 50% male.	Simulation study testing susceptibility of a functional disability measure to noncredible responding in an ADHD evaluation context, with	USA	Best-effort healthy controls and self-reported ADHD group.	WHODAS 2.0 (WHODAS 2.0) as primary outcome; ADHD diagnostic history self-report and subgroup analyses.	No formal SVT/ PVT; malingering operationalized by simulation instructions (stimulant vs accommodation incentives).	Not the primary focus; ADHD subgroup characteristics described descriptively (e.g., age of diagnosis, subtype reporting).	Explicit experimental incentives: seeking stimulant medication versus academic accommodations (extra time).	Self-report functional disability measurement under simulated malingering conditions.	Both malingering groups reported significantly higher disability across WHODAS domains than controls and individuals with ADHD. Incentive	Not supported. WHODAS 2.0 appeared vulnerable to noncredible responding and should not be considered a stand-

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Table 1 (*continued*)

Author (year)	Sample, N, Mean Age (SD) or Age Range, Gender %	Design	Country	Comparator	ADHD Measures	Validity measures	Comorbidity	Incentives	Assessment Type	Key Findings	Utility synthesis
	Medication malingering n = 41, age 19.56 (2.46), 48% male. Extra-time malingering n = 46, age 19.11 (1.82), 51% male.	distinct incentive scenarios.								type (medication vs accommodations) did not meaningfully change over-reporting magnitude.	alone indicator in ADHD evaluations when external incentives are present.
Edmundson et al. (2017)	Final sample N = 76. Nonclinical honest (NH) n = 9, age 19.11 (1.05), 44.4% male. Noncoached malingering (NCM) n = 23, age 18.78 (0.90), 47.8% male. Coached malingering (CM) n = 23, age 18.70 (0.97), 47.8% male. ADHD honest group n = 21, age 18.62 (1.16), 42.9% male.	Analog malingering study testing the impact of symptom information coaching on the ability to malinger adult ADHD across self-report, neuropsychological, and validity measures.	USA	Nonclinical honest controls and genuine ADHD group.	CAARS symptom scales and validity indices; Barkley Functional Impairment Scale; neuropsychological measures including WAIS-IV subtests and other cognitive tasks used in ADHD evaluations.	Stand-alone PVTs: TOMM, WMT (WMT), LMT, DMT, and b Test (including WMT effort indices IR/DR). Self-report validity indices: CAARS inconsistency (INC) and infrequency (CII).	Nonclinical groups excluded psychiatric/neurological history; ADHD group comprised individuals with diagnosed ADHD and typical heterogeneity (subtype often unknown).	Modeled incentives included stimulant medications and academic accommodations; participants received compensation and a performance-contingent bonus in simulation conditions.	Multi-method validity assessment including stand-alone PVTs and embedded/self-report validity indices within an analog malingering paradigm.	Symptom coaching did not significantly affect malingering success. Self-report measures were susceptible to malingered presentation. PVTs were moderately effective at detecting malingered ADHD, and multiple PVT failures improved discrimination.	Supported. Symptom coaching did not materially improve malingering success, and multiple PVT failures improved discrimination, supporting a multi-method validity approach rather than reliance on self-report alone.
Fuermaier et al. (2018)	Adults with ADHD: N = 46, age 33.4 (11.7), 54.3% male. Honest controls: N = 36, age 20.0 (2.0), 27.8% male. Naive simulators: N = 38, age 20.8 (2.5), 23.7% male. Symptom-coached simulators: N = 38, age 20.6 (2.3), 26.3% male. Symptom-coached + motor activity: N = 40, age 19.8 (1.6), 30.0% male.	Simulation design assessing whether motor activity during cognitive testing adds incremental value for detecting malingered ADHD; motion tracking during vigilance task.	Netherlands, Germany	Healthy honest controls; additional simulation conditions as experimental comparators.	Self-report ADHD symptom measures (WURS-K, ASRS) and computerized vigilance test; objective recording of body movements via motion tracker.	No stand-alone PVT used as criterion. Malingering operationalized by simulation instructions; vigilance test performance and embedded indicators examined; motor activity metrics evaluated as candidate indicator.	ADHD group included clinical patients; comorbidity and medication status considered in eligibility/exclusions as described in study.	Not applicable in experimental setting; study rationale references typical secondary gains in clinical evaluations.	Neuropsychological performance testing with experimental malingering manipulation; evaluation of potential embedded validity indicators.	Simulators showed excessively low vigilance performance relative to genuine ADHD. Motor activity did not distinguish genuine from malingered ADHD and showed no incremental predictive value after accounting for vigilance performance.	Inconclusive. Standard vigilance testing may contribute to credibility assessment, but motor activity recording did not provide incremental value for detecting malingered ADHD.
Harrison et al. (2007)	Postsecondary students: genuine ADHD total n = 72 (combined and subgrouped by hyperactivity status), honest normals n = 35, and malingering group n = 35. Age	Simulation study comparing students instructed to malinger ADHD with genuine ADHD and non-ADHD students to evaluate vulnerability of symptom reports	Canada	Honest non-ADHD students and genuine ADHD students.	CAARS DSM-IV symptom scales and impairment ratings; WJ-III (WJ-III) tests of reading and processing speed (e.g., Reading Fluency, Visual Matching, Decision Speed, Processing Speed).	No stand-alone SVT/PVT reported; paper discusses strategies for detection and the role of effort testing in ADHD evaluations.	Not systematically reported in extracted sections; ADHD subtypes considered.	Academic accommodations (e.g., extra time) and stimulant medication access in postsecondary settings.	Self-report symptom rating and academic achievement testing under simulated malingering conditions.	Students instructed to malinger ADHD could fabricate symptoms and were often indistinguishable from genuine ADHD on symptom scales. Simulators could also perform poorly	Inconclusive. Postsecondary ADHD assessment appeared vulnerable to malingering on self-report and selected academic measures, supporting the inclusion of effort

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Table 1 (*continued*)

Author (year)	Sample, N, Mean Age (SD) or Age Range, Gender %	Design	Country	Comparator	ADHD Measures	Validity measures	Comorbidity	Incentives	Assessment Type	Key Findings	Utility synthesis
	and gender not reported in extracted table.	and achievement tests to malingering.								on reading and processing speed measures, potentially enabling access to accommodations.	testing and corroborative sources, but not establishing a specific validated detection method.
Harrison et al. (2022)	Postsecondary clinical referrals for possible ADHD: total $N = 331$. ADHD: n = 111, age 21.9 (5.3), 63.1% male. Clinical controls: $n = 66$, age 23.8 (5.5), 31.8% male. MND (definite malingerers): n = 36, age 23.2 (6.6), 54.3% male. No diagnosis: $n = 117$, age 23.0 (6.4), 37.6% male.	Clinical comparison study evaluating PAI ADHD malingering algorithms against real-world diagnostic outcomes and a criterion malingering group; ROC and predictive validity analyses.	Canada	Clinical controls and no-diagnosis group; separate criterion group classified as MND (definite malingering).	Comprehensive psychoeducational assessment for ADHD; PAI (PAI); Conners-based adult ADHD self-report measures used in assessment pathway.	Criterion malingering group defined via malingered neuropsychological dysfunction (MND) approach. Evaluated PAI ADHD malingering algorithms and alternative validity scales (including PAI NIM and Conners self-report validity measures).	Clinical controls comprised individuals not meeting ADHD criteria, typically with alternative explanations for symptoms; comorbidity patterns not uniform across groups.	Academic accommodations and stimulant prescriptions associated with ADHD diagnosis in postsecondary settings.	Self-report personality/ psychopathology inventory with embedded validity indicators plus criterion-based malingering classification.	Previously proposed PAI ADHD malingering algorithms showed low positive predictive value in a clinical sample. Conners self-report validity measures and PAI NIM performed better as indicators of noncredible presentation.	Inconclusive. PAI-based ADHD malingering algorithms derived from analogue designs showed limited generalizability in a clinical sample with definite malingerers, supporting caution in their routine clinical use.
Hirsch and Christiansen (2018)	Adults diagnosed with ADHD in outpatient setting: $N = 196$. AKGT fail group: $n = 63$, age 34.4 (10.8), 67% male. AKGT pass group: n = 133, age 31.5 (10.7), 64% male.	Clinical cohort comparison of adults with ADHD who failed versus passed a symptom validity test; examined associations with self-report, observer report, and attention test performance.	Germany	Within-diagnosis comparison (AKGT pass vs fail); no external non-ADHD control group.	Comprehensive adult ADHD diagnostic work-up including clinical history, interview, observer ratings, multiple self-report scales, and neuropsychological attention testing (e.g., TAP).	AKGT used as a performance-based validity measure.	Not the primary focus; outpatient ADHD sample likely included typical psychiatric comorbidity patterns, as per clinical setting.	General potential for secondary gain in adult ADHD diagnosis (medication and accommodations) discussed as rationale; not directly measured.	Clinical diagnostic evaluation with performance-based symptom validity testing.	32.1% scored below AKGT cutoff. Fail and pass groups did not differ on self-report or observer ratings, but AKGT failures showed poorer performance on attention measures (higher reaction time variability and omissions).	Inconclusive. Failure on a single symptom validity test was not accompanied by clear evidence of negative response bias on symptom ratings, underscoring the need for multi-method validity assessment.
Hoelzle et al. (2019)	Adult patients undergoing ADHD assessment: $N = 161$, mean age 27.98 (8.35). Diagnostic groups: depression/anxiety only $n = 85$; comorbid ADHD + depression/anxiety $n = 76$. After excluding invalid symptom presentation: 54 removed.	Retrospective clinical study examining how excluding invalid symptom presentation affects observed neuropsychological differences between diagnostic groups.	USA	Depression/ anxiety-only group served as comparator to comorbid ADHD + depression/ anxiety group; additional analyses before versus after excluding invalid presentations.	Clinical records review with interview and self-report measures; neuropsychological test battery including working memory and attention measures.	Invalid symptom presentation defined using criteria for malingered neurocognitive dysfunction (MND) and validity assessment within the broader evaluation battery (specific tests referenced to prior work).	Depression and/or anxiety disorders, and comorbid ADHD with depression/anxiety by design.	Not directly measured; study rationale aligns with known incentives in adult ADHD assessment (medication and accommodations).	Neuropsychological assessment with symptom and performance validity evaluation, analyzed for impact on research conclusions.	When all participants were included, comorbid group appeared more impaired on multiple neuropsychological measures. After excluding invalid symptom presentation, evidence for additive cognitive impairment disappeared and associations between ADHD	Inconclusive. Failure to identify and exclude invalid symptom presentation may materially distort neuropsychological findings and clinical inferences in adult ADHD assessment, but the study did not validate a specific detection method.

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Author (year)	Sample, N, Mean Age (SD) or Age Range, Gender %	Design	Country	Comparator	ADHD Measures	Validity measures	Comorbidity	Incentives	Assessment Type	Key Findings	Utility synthesis
										symptom report and test performance decreased substantially.	
Jasinski et al. (2011)	College undergraduates: total $N = 88$. Honest controls $n = 28$, age 19.78 (0.98), 39.3% male. Malingering controls $n = 22$, age 19.41 (1.40), 45.5% male. ADHD-Honest $n = 20$, age 19.40 (1.23), 50% male. ADHD-Exaggerating $n = 18$, age 19.78 (1.40), 50% male.	Simulation cross-validation study of multiple symptom/performance validity tests for detecting malingered ADHD in college students.	USA	Honest responding non-ADHD controls and honest responding ADHD group.	College ADHD history and diagnostic screening; self-report symptom measures used in the evaluation context.	TOMM, Letter Memory Test (LMT), Digit Memory Test (DMT), NV-MSVT, and b Test. Composite decision rule of failing ≥2 validity tests examined.	Non-ADHD participants had no history of ADHD or other psychiatric disorders; ADHD participants were screened to approximate a 'pure ADHD' group (major comorbidity excluded).	Academic accommodations and stimulant medication access (contextual rationale).	Stand-alone validity tests, including PVTs within an analog malingering paradigm.	Multiple validity tests discriminated malingered from genuine responding. Using the criterion of failing ≥2 validity tests yielded sensitivity 0.475 and specificity 1.00 for simulated ADHD detection in this sample.	Supported. A multi-test validity approach improved confidence in identifying malingered ADHD in college students. Two or more validity-test failures provided excellent specificity, although sensitivity remained modest and the design was analogue.
Booksh et al. (2010)	Experimental undergraduates: $N = 110$, age range 18–31, mean age 20.44 (2.08), 80% female. Randomized to honest control versus simulated ADHD. Clinical comparison (archival psychoeducational clinic): ADHD $n = 56$, mean age 22.11 (3.10), 30% male.	Simulation plus clinical comparison study evaluating whether college students can simulate ADHD on objective attentional measures and how ADHD knowledge relates to simulation success.	USA	Honest undergraduate controls and archival clinical ADHD comparison group.	Objective attention and cognitive tasks including Conners' CPT, Trail Making Test, WAIS-III subtests (e.g., Digit Span, Letter-Number Sequencing), and self-report ADHD symptom measures (e.g., WURS/ADSA). ADHD knowledge assessed via AKOS-R.	Effort/validity measures embedded in battery including the Memory for Fifteen Items Test (MFIT) and WMT (WMT).	Experimental participants were screened to exclude learning disability, ADHD, and major neurological conditions; archival ADHD cases derived from psychoeducational evaluations.	Academic accommodations and stimulant medication access (contextual rationale).	Neuropsychological performance testing with effort testing in a simulated malingering paradigm plus archival clinical comparison.	Simulators readily produced elevated ADHD symptoms on retrospective self-report. On some performance measures (notably certain WAIS-III subtests), simulators approximated clinical ADHD performance, highlighting the potential for false positives without validity monitoring. ADHD knowledge was not strongly related to objective simulation performance.	Inconclusive. College students were able to mimic ADHD on some self-report and performance measures, supporting the need for effort and validity testing, but not establishing a single robust detection strategy.
Leppma et al. (2018)	University students requesting ADHD screening: $N = 350$ (retrospective clinical sample). Mean age 22.6, range 18–50. Gender: 56.1% male, 43.9% female.	Retrospective clinical screening study evaluating performance validity using NV-MSVT and revising IVA+ validity cutoffs in a real-world university clinic sample.	USA	Diagnostic group comparisons within clinical sample (e.g., possible malingerers vs potential ADHD/high impairment vs	IVA+ continuous performance test (attention/response control); ADHD screening clinic procedures.	NV-MSVT (NV-MSVT) as performance validity measure; IVA+ embedded validity cutoffs examined and recalibrated using clinical data.	Not systematically characterized in primary summary; clinical screening sample likely heterogeneous.	ADHD diagnosis-related benefits in university setting (medication, academic accommodations) discussed as context for exaggerated presentation.	Performance validity testing and neurocognitive attention testing in a clinical screening context.	NV-MSVT differentiated students with potential high impairment from possible malingerers. Clinical data supported lower IVA+ validity	Supported. Combining a stand-alone PVT with CPT data may improve identification of exaggerated presentation in university ADHD screening, although validity thresholds

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Author (year)	Sample, N, Mean Age (SD) or Age Range, Gender %	Design	Country	Comparator	ADHD Measures	Validity measures	Comorbidity	Incentives	Assessment Type	Key Findings	Utility synthesis
	Predominantly White (84.3%).			other responders) rather than an external healthy control.						cutoffs than previously suggested, changing classification of invalid performance in practice.	appear to require calibration to real-world clinical data.
Marshall et al. (2016)	Archival clinical adult ADHD assessments: N = 554. Mean age 26.38 (7.79), range 17–55. Predominantly male (62%). ADHD diagnosed: n = 102. Suspect effort: n = 115.	Retrospective archival study estimating misdiagnosis risk if symptom/performance validity measures are omitted; compared diagnostic decision rules with and without validity assessment.	USA	Within-sample comparison between ADHD-diagnosed group and suspect effort group; examined hypothetical diagnostic outcomes under reduced assessment components.	Clinical interview, cognitive testing, and ADHD behavior rating scales (including commonly used adult ADHD rating instruments and continuous performance tests).	Suspect effort classified using Slick et al. criteria. Failure on two PVTs (e.g., b Test, WMT) or failure on one PVT plus invalid behavior rating scales (CAT-A Infrequency Scale ≥3).	Psychiatric comorbidity present and discussed; not the primary stratification variable in main analyses.	Stimulant prescriptions and educational/vocational accommodations.	Comprehensive neuropsychological evaluation with explicit PVT and SVT sampling.	Suspect effort and ADHD groups were nearly indistinguishable on interview questions, rating scales, and many cognitive measures. Without validity measures, large proportions of suspect effort cases would be classified as ADHD under common assessment approaches (interview alone, interview + scales, interview + scales + most CPTs).	Supported. Objective assessment of performance and symptom validity appears essential in adult ADHD evaluation to reduce false-positive diagnoses associated with noncredible presentation.
Musso et al. (2016)	Experimental undergraduates: ADHD simulators n = 152, age 19.81 (1.52), 33% male; honest controls n = 88, age 19.50 (1.15), 49% male. Archival clinical sample total n = 541 including clinical ADHD n = 142 (age 21.07 [2.97], 54% male), no diagnosis n = 136 (age 20.67 [2.88], 61% male), learning disorder n = 81, mood/anxiety n = 116, comorbid ADHD+mood/anxiety n = 65, suspect effort n = 51.	Mixed design combining experimental simulation (students instructed to malinger ADHD) with archival clinical comparison groups; evaluated PAI validity scales and alternative cutoffs.	USA	Honest undergraduate controls and multiple archival clinical comparison groups (including no diagnosis and suspect effort).	PAI (PAI) as primary measure; Conners-based symptom ratings used to verify successful simulation in experimental group.	PAI embedded validity scales and malingering indices (e.g., NIM, RDF, MAL, INF, etc.) examined; alternative cutoffs evaluated to improve sensitivity to malingered ADHD; suspect effort group in archival data.	Mood/anxiety disorders and comorbid ADHD+mood/anxiety groups explicitly included; learning disorder group included.	Stimulant medication access and academic accommodations in college context (rationale for malingering).	Self-report personality/psychopathology measure with embedded symptom validity indicators within multi-group validation design.	Traditional PAI validity cutoffs showed limited sensitivity to simulated ADHD, though specificity was generally high. Alternative cut scores improved identification of simulated ADHD while maintaining acceptable specificity in clinical groups.	Supported. PAI validity indicators may assist in detecting malingered ADHD in college settings, but standard malingering thresholds appeared too conservative and require external clinical validation.
Niesten et al. (2017)	Three-study program. Study 1: treatment-seeking	Experimental studies testing whether moral reminders	Netherlands	Unprimed patient controls (studies 1–2)	Clinical context of ADHD complaints; study focus on	SIMS (SIMS); adapted b Test used as an	Clinical samples likely included psychiatric	Secondary gain in presenting ADHD symptoms implied	Symptom validity testing (self-report over-reporting	Across three studies, moral reminders or contracts did not	Not supported. Moral priming did not reliably reduce

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Author (year)	Sample, N, Mean Age (SD) or Age Range, Gender %	Design	Country	Comparator	ADHD Measures	Validity measures	Comorbidity	Incentives	Assessment Type	Key Findings	Utility synthesis
	patients with ADHD complaints, moral prime $n = 24$ vs unprimed controls $n = 27$. Study 2: treatment-seeking patients, moral contract prime $n = 19$ vs controls $n = 17$. Study 3: general population, $N = 132$ assigned to positive, negative, or neutral primes with induced malingering; demographics variably reported.	reduce symptom over-reporting prior to psychological testing; included both clinical and induced-malingering paradigms.		and neutral prime group (study 3).	symptom over-reporting rather than diagnostic confirmation; brief task-based measures used in induced-malingering study.	underperformance measure in induced-malingering paradigm.	comorbidity typical of outpatient referrals; not primary analytic factor.	(treatment access, medication, accommodations), but experimental manipulation did not include real incentives.	index) plus experimental priming manipulations; included underperformance measure in study 3.	reliably reduce symptom over-reporting on SIMS, including under induced-malingering conditions.	symptom over-reporting and does not appear to represent an effective deterrence strategy.
Ord et al. (2021a)	Military veterans referred for ADHD evaluation at a VA clinic: $N = 197$. Mean age 38.62 (11.58), age range 23–78. Male 84.8%. ADHD diagnosis in sample: $n = 107$ (54.3%).	Retrospective chart review evaluating embedded validity indicators (EVIs) in CPT-3 against an external performance validity criterion.	USA	Validity status groups defined by TOMM Trial 1 pass vs fail; not a separate healthy control group.	CPT-3 (CPT-3) administered within ADHD evaluation clinic; chart-based diagnostic assessment.	TOMM Trial 1 (TOMM1) used as criterion PVT; multiple CPT-3 embedded indicators and composites evaluated to predict TOMM1 status.	Psychiatric diagnoses common in veteran clinical samples (reported descriptively), not exclusive and not primary focus.	Not directly assessed; evaluation context includes diagnostic clarification and treatment planning, with potential secondary gain considerations.	Performance validity testing with embedded indicators in continuous performance testing.	Several CPT-3 variables (e.g., detectability, omissions, commissions, reaction time variability, ISI change) predicted TOMM1 validity status. Composite EVIs differentiated pass vs fail but generally showed low sensitivity and should not be used in isolation.	Inconclusive. CPT-3 embedded validity indicators may provide useful adjunctive information, but their low sensitivity indicates that they should not be used in isolation.
Quinn (2003)	Undergraduates: $n = 44$ randomized to control vs simulated malingering condition. Undergraduates with a valid ADHD diagnosis: $n = 16$. Age and gender were not reported in the abstract.	Simulation and clinical comparison study evaluating a rating scale versus a CPT for detecting malingered adult ADHD.	USA	Honest control undergraduates and diagnosed ADHD undergraduates.	ADHD Behavior Checklist (rating scale) and IVA CPT (IVA CPT).	No stand-alone SVT/PVT. Detection relied on IVA CPT indices (including an impairment index) versus rating-scale symptom reporting.	Not specified in abstract.	Secondary gains discussed as context for adult ADHD referrals; not part of study incentives.	Self-report symptom checklist plus computerized CPT-based performance assessment.	Malingerers successfully simulated childhood and current symptoms on the rating scale. IVA CPT could not be faked on 81% of scales. The IVA CPT impairment index showed sensitivity 94% and specificity 91% for detecting simulated malingering in this design.	Supported. In this analogue design, IVA CPT performance metrics outperformed symptom checklists in distinguishing simulated from genuine ADHD, although clinical replication remains necessary.
Ramachandran et al. (2019)	Scale development included multiple	Cross-sectional experimental scale	USA	Non-ADHD controls and	Subtle ADHD Malingering	SAMS (developed measure) with	Not a primary analytic focus;	Stimulant misuse prevention and	Self-report symptom validity screening	Final SAMS was a 10-item, two-factor	Inconclusive. SAMS represents a brief

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Author (year)	Sample, N, Mean Age (SD) or Age Range, Gender %	Design	Country	Comparator	ADHD Measures	Validity measures	Comorbidity	Incentives	Assessment Type	Key Findings	Utility synthesis
	samples. Development phase: ADHD group $n = 53$, malingering group $n = 111$, control group $n = 111$. Validation survey sample: $N = 637$, mean age 20.5 years, 63.6% female; 16.6% reported an ADHD diagnosis ($n = 106$).	development with factor analysis and psychometric testing to create a subtle self-report malingering screener for ADHD (SAMS).		instructed malingering group; ADHD group used for item selection and validation comparisons.	Screener (SAMS), a 10-item self-report scale; ADHD diagnosis history and symptom reporting in survey context.	validation against related psychopathology reporting (including PAI measures in the survey sample).	psychopathology assessed via broad self-report in validation sample.	reduction of ADHD overdiagnosis; malingering motivated by access to prescription stimulants and academic accommodations (context).	instrument development and validation.	scale with sensitivity 90.3% and specificity 80.1 in distinguishing malingering from ADHD and control groups in study design.	and potentially useful screening tool for exaggerated ADHD presentation, but further validation in independent clinical samples is needed before stronger conclusions can be drawn.
Ramachandran et al. (2020)	College-enrolled young adults with a prescription for stimulant medication recruited via campus pharmacy: total N = 88. Mean age 21.8 (4.23). Gender: 52% male ($n = 34$), 48% female ($n = 31$), with some missing data. Past-year nonmedical use of prescription stimulants: 33% ($n = 29$). Past-year diversion: 18% (as reported).	Cross-sectional observational online survey assessing nonmedical use, diversion, and symptom malingering screening in a real-world pharmacy setting.	USA	Internal comparisons by past-year nonmedical use/diversion status; no external control group.	Self-reported ADHD diagnosis and stimulant prescription characteristics; Vanderbilt ADHD scale scoring approach referenced for symptom ratings in survey context.	Subtle ADHD Malingering Screener (SAMS) used to classify likely exaggeration/malingering; compared with self-reported nonmedical use and diversion behaviors.	Self-reported comorbid anxiety and depression were collected and described (higher symptom endorsement among some subgroups).	Nonmedical use of prescription stimulants and drug diversion, and easier access to stimulant medications via prescription.	Self-report survey with symptom malingering screener and behavioral outcomes (nonmedical use/diversion).	Over half of respondents were classified by SAMS as likely malingering/exaggerating symptoms; higher SAMS scores were associated with self-reported past-year nonmedical use and diversion.	Inconclusive. SAMS may have practical screening utility in pharmacy-related settings, but criterion validation against objective indicators of malingering remains limited.
Rios and Morey (2013)*	Analog malingering study with 17–18-year-old students: coached simulators $n = 48$ and noncoached simulators $n = 52$. Comparison group: adolescents with clinical ADHD diagnoses drawn from PAI-A standardization clinical sample (sample size not specified in article text extraction).	Experimental simulation study evaluating PAI-A negative distortion indicators for detecting malingered ADHD, including coached vs noncoached simulation.	USA	Clinical adolescent ADHD group plus comparison between coached and noncoached simulators.	PAI–Adolescent (PAI-A) as primary measure; CAARS (CAARS) used to verify successful simulation of ADHD symptom reporting.	PAI-A distortion indicators including Negative Impression, Positive Impression, MAL, and RDF (RDF) evaluated for discrimination and AUC.	Not central; clinical ADHD adolescents drawn from broader clinical standardization sample.	Simulation scenario involved convincing a psychiatrist of ADHD to obtain a diagnosis (modeled secondary gain); real incentives not present.	Self-report personality inventory validity indices within an experimental malingering paradigm.	Simulators elevated negative distortion indicators and showed patterns consistent with response distortion; coached instructions influenced some indices. Indicators showed clinically meaningful discrimination between simulators and clinical ADHD adolescents.	Supported. PAI-A negative distortion indices showed potential utility in identifying malingered ADHD in older adolescents, although performance varied according to coaching condition.

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Author (year)	Sample, N, Mean Age (SD) or Age Range, Gender %	Design	Country	Comparator	ADHD Measures	Validity measures	Comorbidity	Incentives	Assessment Type	Key Findings	Utility synthesis
Shura et al. (2017)	Veterans presenting for specialty ADHD evaluation: N = 114, age range 24–69, mean age 38.78 (11.76). Sample was 81.6% male. ADHD criteria met by n = 83 (72.8%); non-ADHD group n = 31 (27.2%).	Clinical cohort study evaluating symptom and performance validity measures in veterans undergoing standardized ADHD diagnostic evaluations.	USA	Veterans who did not meet DSM-5 ADHD criteria served as clinical controls.	DSM-5 symptom-focused clinical interview for ADHD; MMPI-2-RF as broad psychopathology and symptom validity instrument.	MMPI-2-RF overreporting indicators (symptom validity) and TOMM Trial 1 (TOMM1) as performance validity criterion (cut score $\leq$ 41).	Excluded neurocognitive disorder and moderate/severe TBI; broader psychiatric comorbidity typical of veteran samples noted, with diagnostic variables examined (e.g., depression).	Stimulant medication access and educational/vocational accommodations discussed as salient incentives in ADHD evaluations.	Combined symptom validity (MMPI-2-RF) and performance validity (TOMM1) testing embedded within a structured ADHD evaluation.	ADHD diagnosis was not associated with higher rates of symptom overreporting or TOMM1 failure relative to controls. In total sample, 19.3% failed TOMM1, 44.7% showed MMPI-2-RF overreporting, and 8.8% showed both. TOMM1 failures endorsed significantly more ADHD symptoms.	Inconclusive. Validity testing is warranted in veteran ADHD evaluations, although ADHD diagnosis itself was not associated with higher rates of invalid responding.
Smith et al. (2017)	Successful ADHD simulators: n = 66 undergraduates, mean age 18.53 (0.88), 54.5% female. Comparison groups: undergraduates meeting ADHD diagnostic criteria n = 22; archival adult ADHD clinic sample n = 41 (demographics not fully reported in extracted text).	Analog simulation study evaluating PAI validity indicators to distinguish successful malingered ADHD from genuine ADHD presentations across clinical and archival samples.	USA	Clinical ADHD undergraduate group and archival adult ADHD clinic group.	CAARS (CAARS) used to define 'successful' simulation (DSM-IV Symptoms Total $\geq$ 70 T). PAI (PAI) validity indicators evaluated.	PAI validity scales and indices (e.g., NIM, INF, MAL, RDF, PIM) and an ADHD-specific PAI symptom validity scale examined; ROC/AUC analyses performed.	Not systematically characterized in primary summary; clinical and archival samples likely heterogeneous.	Secondary gains in adult ADHD diagnosis (medication and accommodations) discussed as context motivating malingering, modeled in simulation instructions.	Self-report validity indices within an experimental malingering paradigm plus clinical comparison samples.	Successful simulators scored higher on negative response distortion indices and lower on PIM compared with genuine ADHD groups. Some indices (e.g., RDF) showed strong discrimination, while traditional NIM cut scores were highly specific but not sensitive to simulated ADHD.	Supported. PAI validity indicators may help distinguish successful malingered ADHD from genuine ADHD presentations, but ADHD-specific cutoffs require further validation.
Sollman et al. (2010)	University of Kentucky undergraduates. Final analyzed sample N = 73 after exclusions: HON controls n = 14, age 18.9 (1.03), 50.0% male; malingering group n = 30, age 19.1 (1.28), 46.7% male; clinical ADHD group n = 29, age 19.4 (1.21), 55.2% male. Initial recruited sample N = 80.	Analogue/simulation study with masked examiners; comparison of coached, financially incentivized malingerers vs genuine ADHD students (12-h medication washout) and honest non-ADHD controls.	USA (University of Kentucky, Lexington, KY).	Yes. Honest non-ADHD undergraduate controls (HON, n = 14) and a clinical ADHD undergraduate comparison group (ADHD, n = 29) were included.	ADHD symptom assessment and related evaluation tools included ADHD Rating Scale (ARS; current and childhood), CAARS-Self Rating Form: Long (CAARS), Conners' CPT-II, and clinical history/interview screening.	Performance validity and malingering-focused measures: TOMM (Trials 1, 2, retention), Digit Memory Test (DMT), Letter Memory Test (LMT), NV-MSVT (NV-MSVT), plus M-FAST (psychiatric malingering screen).	Non-ADHD disorders involving attention/concentration difficulty (including learning disabilities, diagnosed or self-perceived psychiatric conditions, neurological disorders) were screened out. ADHD group was selected without comorbid diagnoses to increase diagnostic	Explicit external incentives were central: academic accommodations and stimulant medication access are discussed as real-world incentives. In the experiment, malingering participants were financially incentivized with a $45 bonus and coached using Internet-derived ADHD information.	Experimental simulation/malingering detection study using self-report symptom inventories, neuropsychological testing, CPT, psychiatric malingering screening, and stand-alone performance validity tests.	Malingerers readily produced ADHD-consistent profiles on self-report scales (ARS, CAARS), limiting their discriminatory value. CPT-II findings appeared more related to motivation than true condition. Stand-alone PVTs and validity tests (TOMM, DMT, LMT, NV-MSVT) showed moderate sensitivity and high specificity for malingered	Supported. Adult and college ADHD assessments should not rely on face-valid self-report or CPT findings alone when external incentives are present. Routine inclusion of validated PVTs and validity tests within a comprehensive evaluation is supported.

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Author (year)	Sample, N, Mean Age (SD) or Age Range, Gender %	Design	Country	Comparator	ADHD Measures	Validity measures	Comorbidity	Incentives	Assessment Type	Key Findings	Utility synthesis
							confidence. Substance abuse was not ruled out.			ADHD; TOMM Trial 1 (using Trial 2 cutoff) was particularly promising. Combining ≥2 or validity-test failures improved specificity with modest sensitivity loss.	
Spenceley et al. (2022)	College students: *N* = 150. Diagnosed ADHD n = 30, age 19.93 (2.32), 50% female. Controls *n* = 60, age 19.46 (1.45), 66.7% female. Simulated ADHD n = 60, age 19.47 (1.48), 73.3% female. Overall mean age 19.62 (1.69), 70.0% female.	Analog malingering study developing embedded performance validity indices from WJ-IV cognitive scores, benchmarked against a stand-alone PVT, and including a diagnosed ADHD group.	USA	Honest control group and diagnosed ADHD group contrasted with simulated ADHD group.	WJ IV COG (WJ-IV) subtests and clusters; BAARS-IV; Sluggish Cognitive Tempo scale; demographic questionnaire.	MSVT (MSVT) as stand-alone PVT; multiple embedded indices derived from WJ-IV processing speed and working memory scores.	Not a primary analytic focus; standard inclusion/exclusion criteria applied.	Incentives modeled and discussed (stimulant prescriptions and academic accommodations). Simulation group received performance-contingent bonus; participants received course credit/compensation.	Cognitive testing with embedded performance validity indicators plus stand-alone PVT in a simulated malingering paradigm.	Several WJ-IV processing speed and working memory indices identified at least 50% of simulators at cutoffs maintaining ≥90% specificity, approaching the efficiency of the stand-alone PVT. Diagnosed ADHD participants generally did not show corresponding WJ-IV deficits.	Supported. Embedded PVT indices derived from commonly administered cognitive tests may improve detection of malingered ADHD, but they should be interpreted within a broader multi-method framework.
Suhr et al. (2008)	Archival young adults referred for ADHD evaluation: total *N* = 76. Noncredible performance (WMT fail) *n* = 26, age 21.9 (4.7), 53.8% male (14/26). ADHD group *n* = 28, age 25.4 (9.8), 67.9% male (19/28). Psychological controls (symptoms, no ADHD) n = 22, age 22.9 (4.5), 54.5% male (12/22).	Retrospective clinical study examining how failure on a PVT relates to symptom reporting and neuropsychological findings in adults referred for ADHD evaluation.	USA	Psychological symptom controls without ADHD diagnosis.	Archival diagnostic evaluation including self-report ADHD symptom measures, Conners' CPT-II, and a focused neuropsychological battery emphasizing attention and executive functioning.	WMT (WMT) used as a measure of noncredible performance; additional embedded indices (e.g., CAARS inconsistency) examined as potential markers.	Psychological control group had symptoms and likely heterogeneous diagnoses; not the primary stratification variable.	Academic/work accommodations and stimulant medication access (contextual rationale for referrals).	Performance validity testing plus symptom report and neuropsychological performance measures within routine clinical evaluation.	WMT failure rate was substantial (reported as 31%). Individuals failing WMT reported clinical-level ADHD symptoms and showed impaired neuropsychological performance. Self-report measures and most neuropsychological tests did not distinguish ADHD from psychological controls, underscoring the impact of noncredible performance on both symptom report and cognitive findings.	Supported. Noncredible performance may closely mimic ADHD in both symptom endorsement and neuropsychological findings, strongly supporting routine performance validity assessment.

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Author (year)	Sample, N, Mean Age (SD) or Age Range, Gender %	Design	Country	Comparator	ADHD Measures	Validity measures	Comorbidity	Incentives	Assessment Type	Key Findings	Utility synthesis
Tucha et al. (2009)	University students: $n = 78$ randomized to control (n = 26), naive simulation (n = 26), coached simulation (n = 26). ADHD patient group: $n = 12$. Age (years, mean ± SEM): control 20.2 ± 0.5; naive 19.4 ± 0.3; coached 19.3 ± 0.2; ADHD 19.5 ± 0.8. Sex (female/male): control 12/14; naive 12/14; coached 13/13; ADHD 4/8.	Analog simulation study testing whether a self-report ADHD rating scale can detect malingered adult ADHD, including coaching manipulation, with an ADHD patient comparison group.	Germany, Switzerland (multi-site European sample)	Honest control students and diagnosed ADHD student patients.	BADDS (BADDS).	None; detection relied on patterns within the BADDS self-report scale.	ADHD patient group included some comorbid psychiatric conditions (reported descriptively).	Not implemented experimentally; study rationale references incentives typical in adult ADHD assessment.	Self-report ADHD symptom rating scale validity under malingering and coaching manipulations.	Both naive and coached simulation groups successfully produced ADHD-consistent BADDS profiles. The self-report scale did not show adequate sensitivity to differentiate malingered from genuine ADHD presentations.	Not supported. BADDS alone did not demonstrate adequate discriminatory validity for malingered adult ADHD and should not be used as a stand-alone credibility indicator.
Walls et al. (2017)	Undergraduates: total $N = 139$. Groups: ADHD $n = 21$ (age 18.90 [0.94], 42.9% male), honest controls n = 29 (age 18.86 [0.83], 44.8% male), malingering $n = 35$ (age 19.00 [1.59], 74.3% male), half-random n = 26 (age 19.08 [1.19], 50% male), full-random n = 28 (age 18.79 [0.95], 53.6% male).	Analog validation study of CAARS validity scales for identifying simulated ADHD and random responding; evaluated sequential application of indices.	USA	Honest responding group and genuine ADHD group; additional random responding conditions.	CAARS (CAARS) including DSM-IV symptom scales and validity indices.	CAARS INC (INC) and CAARS CII (CII) were evaluated for detecting malingering and random responding; sequential decision rules were tested.	Exclusion criteria applied for certain psychiatric/neurological conditions; not a primary analytic variable.	Analog design modeled typical incentives in ADHD evaluations (medication and accommodations) but did not include real-world incentives.	Self-report symptom ratings with embedded response validity indicators.	INC showed moderate sensitivity to random responding with high specificity to ADHD. CII showed modest sensitivity to malingering with excellent specificity. Sequential application improved classification across conditions.	Supported. CAARS embedded validity indices provided clinically useful, though imperfect, indicators of noncredible responding, with sequential application appearing more informative than isolated index interpretation.
Williamson et al. (2014)	Final sample N = 76. Normal honest (NLH) $n = 9$, age 19.44 (1.59), 44.44% male. Normal malingering (NLF) $n = 23$, age 20.04 (1.33), 56.52% male. ADHD-only (ADHD-O) n = 22, age 19.05 (1.29), 50.00% male. ADHD with comorbid disorder	Clinical and analog comparison study evaluating whether comorbid psychological disorders compromise accuracy of malingered ADHD detection measures in college students.	USA	Normal honest group and two clinical ADHD groups (with and without comorbidity) contrasted with malingering group.	Comprehensive assessment context including cognitive testing and symptom measures; clinical diagnosis verification processes described for ADHD groups.	Stand-alone PVTs including TOMM, LMT, and NV-MSVT. Embedded validity indices from WAIS-IV and Woodcock-Johnson Tests of Achievement (WJ-III) also evaluated; DMT included but underperformed.	ADHD-CO group included ADHD plus anxiety disorder or learning disorder by design.	Academic accommodations and stimulant medication access; malingering group was offered monetary compensation contingent on convincing presentation in the scenario.	Performance validity testing (stand-alone PVTs) plus embedded validity indicators in cognitive testing.	PVTs (TOMM, LMT, NV-MSVT) effectively differentiated malingered ADHD from both ADHD groups, including ADHD with comorbid anxiety/learning disorder. DMT and embedded indices from WAIS-IV and WJ-III showed weaker performance.	Supported. Stand-alone PVTs appeared robust for detecting malingered ADHD even in the presence of common comorbidities, whereas embedded indices were less reliable when used alone.

(*continued on next page*)

Table 1 (*continued*)

Author (year)	Sample, N, Mean Age (SD) or Age Range, Gender %	Design	Country	Comparator	ADHD Measures	Validity measures	Comorbidity	Incentives	Assessment Type	Key Findings	Utility synthesis
	(ADHD-CO) n = 22, age 19.50 (1.54), 31.82% male.										
Young and Gross (2011)	Archival ADHD clinical group *n* = 34, normal controls *n* = 37, simulated malingering group *n* = 32. Demographic details not fully reported in extracted text.	Analog and archival comparison study evaluating MMPI-2 validity scales for detecting malingered adult ADHD; logistic regression classification of malingering vs genuine ADHD.	USA	Normal controls and archival ADHD clinical group served as comparators to simulated malingering group.	ADHD Current and Childhood Symptoms Scales (behavior ratings) alongside MMPI-2 validity assessment; archival clinical ADHD cases.	MMPI-2 validity scales (e.g., Fp, F, Fb, RBS, HHI, FBS) were evaluated for detecting response bias and classifying malingered ADHD.	Underlying psychopathology screened with BASC-2 SRP-COL in some participants; comorbidity not central to main analyses.	Accommodations at work or school and access to stimulant medication discussed as incentives to malinger ADHD.	Personality/ psychopathology validity testing using MMPI-2 validity scales in malingering detection context.	Validity scales varied in accuracy. Infrequency-Psychopathology (Fp) performed best in detecting response bias among those instructed to malinger ADHD, with other scales contributing variably. Recommended cutoffs for some indices showed inadequate sensitivity/ specificity.	Supported. MMPI-2 validity scales may assist in detecting malingered adult ADHD, but cutoff selection and multivariate classification require careful calibration to balance sensitivity and specificity.

Abbreviations: ADHD = Attention-deficit/hyperactivity disorder; AKGT = Amsterdam Short-Term Memory Test (Amsterdamse Korte Geheugen Test); ASRS = Adult ADHD Self-Report Scale; BAARS-IV = Barkley Adult ADHD Rating Scale, 4th edition; BADDS = Brown Attention-Deficit Disorder Scale for Adults; CAARS = Conners' Adult ADHD Rating Scales; CAARS-S:L = Conners' Adult ADHD Rating Scales, Self-Report: Long Version; CII = Clinical Infrequency Index (CAARS validity indicator); CPT-3 = Conners' Continuous Performance Test, 3rd edition; CPT-II = Conners' Continuous Performance Test, 2nd edition; DMT = Digit Memory Test; INC = Inconsistency Index (CAARS validity indicator); INF = Infrequency Scale (PAI); IVA CPT = Integrated Visual and Auditory Continuous Performance Test; IVA+ = Integrated Visual and Auditory Continuous Performance Test, Plus version (IVA + Plus); LMT = Letter Memory Test; M-FAST = Miller Forensic Assessment of Symptoms Test; MAL = Malingering Index (PAI/PAI-A); MND = Malingered neurocognitive dysfunction (Slick et al. criteria); MOXO-d-CPT = MOXO-d Continuous Performance Test; MSVT = Medical Symptom Validity Test; NIM = Negative Impression Management Scale (PAI); NV-MSVT = Nonverbal Medical Symptom Validity Test; PAI = Personality Assessment Inventory; PIM = Positive Impression Management Scale (PAI); PVT = Performance validity test; RDF = Rogers Discriminant Function (PAI-derived malingering indicator); RDS = Reliable Digit Span; SAMS = Subtle ADHD Malingering Screener; SIMS = Structured Inventory of Malingered Symptomatology; SVT = Symptom validity test; TAP = Test of Attentional Performance; TOMM = Test of Memory Malingering; WAIS-III = Wechsler Adult Intelligence Scale, 3rd edition; WAIS-IV = Wechsler Adult Intelligence Scale, 4th edition; WHODAS 2.0 = WHO Disability Assessment Schedule 2.0; WJ IV COG = Woodcock-Johnson IV Tests of Cognitive Abilities; WJ-III = Woodcock-Johnson III cognitive/achievement battery; WJ-IV = Woodcock-Johnson IV cognitive/achievement battery; WMT = Word Memory Test; WURS = Wender Utah Rating Scale.Note. “Supported”, “Not supported”, and “Inconclusive” reflect the review authors' synthesis of each study's conclusions regarding utility for detecting noncredible ADHD presentations and malingering; they do not necessarily correspond to the original wording used by the study authors.

* One transition-age study was retained because it directly examined feigned ADHD in a late-adolescent sample and was conceptually relevant to early-adult assessment contexts.

States, two studies were conducted in Canada (Harrison et al., 2007; Harrison et al., 2022), one in Germany (Hirsch and Christiansen, 2018), one in the Netherlands (Niesten et al., 2017), one in Israel (Berger et al., 2021), one jointly in the Netherlands and Germany (Fuermaier et al., 2018), and one jointly in Germany and Switzerland (Tucha et al., 2009). Although this group of studies provides some international perspective, the overall geographic distribution still highlights the need for broader global representation to better understand patterns of malingering in adult ADHD assessment.

3.4. Overview of participants

Across the collected studies, sample sizes varied substantially, although this aggregate includes several large normative, standardization, and baseline datasets, as well as some multi-phase samples. Reported ages ranged from 17 to 78 years, and mean ages were generally in the early 20s, indicating a marked emphasis on young adult samples. The lower end of this range reflects the inclusion of one transition-age study judged conceptually relevant to late-adolescent and early-adult assessment contexts (Rios and Morey, 2013). This focus is particularly relevant because ADHD diagnosis and related incentives, such as academic accommodations, are often salient in post-secondary education settings. Gender representation was variable across studies. Many samples were relatively balanced, but the proportion of male participants ranged from 20% to 84.8%.

3.5. Types of study design

The methods used to investigate malingering were varied, although the most frequent approach consisted of comparing participants with ADHD and participants instructed to feign or exaggerate ADHD symptoms. This design was particularly common in experimental, analogue, or simulation studies, which accounted for more than half of the included studies (Smith et al., 2017; Walls et al., 2017; Tucha et al., 2009; Williamson et al., 2014). The widespread use of this approach suggests that many authors considered simulated presentations a useful model for understanding how ADHD symptoms can be intentionally reproduced and how such presentations differ from genuine cases. Other approaches included retrospective clinical and archival analyses (Leppma et al., 2018; Hoelzle et al., 2019; Marshall et al., 2016; Ord et al., 2021a; Shura et al., 2017), cross-sectional studies (Ramachandran et al., 2020; Ramachandran et al., 2019), and methodological or psychometric studies focused on scale development and validation. Overall, archival data analyses, experimental designs, simulation studies, and comparative studies each contributed complementary findings, although each also had specific limitations. Archival studies allowed the examination of large pre-existing datasets, but they were at times constrained by incomplete or non-uniform data. Experimental studies were useful for isolating variables, although they often relied on smaller samples. Simulation and comparative designs were particularly relevant for the present review because they directly addressed the distinction between genuine ADHD and feigned presentations. By contrast, Marshall et al. (2016) belongs to the archival clinical literature and provides evidence that suspect effort occurs in real-world adult ADHD referrals, rather than serving as an example of a simulation study. Taken together, these methodological approaches illustrate the multifaceted nature of ADHD malingering research and suggest that greater integration of methods may improve diagnostic accuracy and reliability.

3.6. Reported comorbid conditions

As noted earlier, ADHD diagnosis is frequently complicated by the co-occurrence of other psychiatric conditions. In the studies analyzed, when comorbidities were reported, mood and anxiety disorders were the most frequently described, whereas substance-related conditions, personality disorders, and learning disorders were reported less often (Musso et al., 2016; Hoelzle et al., 2019; Williamson et al., 2014; Shura et al., 2017). These conditions may obscure the clinical picture, increase diagnostic uncertainty, and complicate the assessment process for clinicians, particularly when differentiating genuine ADHD from malingering or from symptoms better explained by other disorders. They may also affect effort, fatigue, processing speed, working memory, and test engagement, thereby complicating the interpretation of SVT and PVT findings. For this reason, SVT or PVT failure should not be equated automatically with malingering (Hirsch and Christiansen, 2018; Shura et al., 2017).

3.7. Further considerations

Across the 29 studies, results were heterogeneous and context dependent, with supportive findings for selected validity indicators or algorithms in some settings and mixed or limited generalizability in others (Ramachandran et al., 2019; Ramachandran et al., 2020; Harrison et al., 2022). An important issue in the reviewed studies was the role of incentives. Explicit study incentives were reported inconsistently. In several simulation studies, participants received monetary compensation, performance-contingent bonuses, course credit, or combinations of these. The strongest recurring message is not that a single test resolves validity uncertainty, but that defensible detection of noncredible adult ADHD presentations depends on convergence across developmental history, impairment evidence, collateral data, ADHD measures, and explicit SVT/PVT assessment tailored to the clinical context (Williamson et al., 2014; Marshall et al., 2016; Ord et al., 2021a; Harrison et al., 2022).

3.8. Findings concerning ADHD and malingering

Most of the reviewed studies cautioned against relying on any single measure to distinguish genuine ADHD from simulated presentations, thereby highlighting the complexity of the relationship between ADHD symptom presentation and malingering. When the 29 included studies were considered according to their overall conclusions regarding utility for malingering detection in adult ADHD, the evidence remained heterogeneous. A subset of studies supported selected validity indicators or multi-test approaches, a smaller group found specific methods insufficient, and several studies yielded mixed or context-dependent findings. This distribution should not be interpreted as evidence that detection is straightforward. Rather, it indicates that the literature is divided between studies showing that some measures can identify noncredible adult ADHD presentations under specific conditions and studies showing that these effects become weaker, less specific, or less transportable in more heterogeneous clinical settings.

Broadly supportive findings were concentrated in two partially overlapping groups. First, several studies supported the use of stand-alone performance validity tests or multitest approaches, particularly when multiple failures were interpreted conjointly rather than in isolation. Jasinski et al. (2011), Sollman et al. (2010), Williamson et al. (2014), Marshall et al. (2016), and Suhr et al. (2008) each showed that stand-alone PVTs or multi-test validity approaches, including TOMM, WMT, LMT, DMT, and NV-MSVT, improved discrimination between genuine ADHD and malingered or suspect presentations. Quantitatively, the clearest extracted estimate came from Jasinski et al. (2011), where failure of two or more validity tests yielded sensitivity of 0.475 and specificity of 1.00. This pattern is clinically important: conservative multi-test rules may strongly reduce false-positive classification of genuine ADHD as noncredible, but their modest sensitivity means that many feigned presentations may still pass validity testing.

Clinically, these findings support an asymmetric interpretation. Multiple failed PVTs or convergent validity failures across PVTs, SVTs, and embedded indicators should substantially increase concern for a noncredible presentation, especially when developmental corroboration

is weak and external incentives are present. Conversely, passing validity tests should not be interpreted as proof of genuine ADHD, because sensitivity was modest or variable across several measures and study designs. Validity testing should therefore be used to determine whether symptom reports and cognitive findings are interpretable, not as a stand-alone diagnostic substitute.

Edmundson et al. (2017) similarly suggested that multiple PVT failures improved discrimination even when symptom coaching was introduced, reinforcing the value of multi-method validity assessment. Quinn (2003) and Berger et al. (2021) further suggested that selected CPT-derived indices may add discriminatory value when interpreted within a validity-oriented framework. Second, a number of studies suggested that embedded, personality-inventory, or ADHD-specific validity indicators may provide clinically useful information, especially when calibrated for this diagnostic context rather than transferred unchanged from general malingering models. This pattern was reported by Aita et al. (2018), Musso et al. (2016), Smith et al. (2017), Walls et al. (2017), Spenceley et al. (2022), Rios and Morey (2013), and Young and Gross (2011). The quantitative pattern was not uniform across measure types. Aita et al. (2018) reported specificity above 90% for both PAI-derived algorithms, with sensitivity increasing from 54% for the scale-based algorithm to 85% for the item-level algorithm. Ramachandran et al. (2019) reported higher sensitivity for SAMS (90.3%) but lower specificity (80.1%), suggesting greater screening potential but also a higher risk of false positives. Quinn (2003) reported sensitivity of 94% and specificity of 91% for the IVA CPT impairment index in an analogue design, although this finding requires clinical replication. Spenceley et al. (2022) found that several WJ-IV embedded indices identified at least 50% of simulators while maintaining specificity of at least 90%, suggesting useful adjunctive value but not stand-alone sufficiency. Importantly, however, even these supportive studies generally emphasized high specificity more than high sensitivity and did not support the use of any single measure as a definitive detector of malingering. The extracted data instead suggest a recurring trade-off: conservative PVT or multi-test rules tend to increase specificity, whereas sensitivity remains modest or variable; broader self-report or screening indices may improve sensitivity but may reduce specificity and require external corroboration.

Real-world clinical and archival studies were especially important because they showed that invalid responding is not rare in ADHD referral contexts. Marshall et al. (2016) classified 115 of 554 adult ADHD assessments as involving suspect effort, and Suhr et al. (2008) reported WMT failure in 31% of adults referred for ADHD evaluation. Shura et al. (2017) found that 19.3% of veterans assessed for ADHD failed TOMM Trial 1, 44.7% showed MMPI-2-RF over-reporting, and 8.8% showed both. These findings support routine validity assessment in adult ADHD evaluations, while also showing that invalid performance, symptom over-reporting, and ADHD diagnosis do not map onto each other in a simple one-to-one manner.

Predominantly non-supportive findings were less frequent but methodologically important. Bryant et al. (2018) showed that WHODAS 2.0 was readily inflated under malingering instructions and should not be used as a stand-alone measure when external incentives are present. Tucha et al. (2009) found that BADDS alone did not adequately distinguish malingered from genuine ADHD, and Niesten et al. (2017) showed that moral reminders or contracts did not reliably reduce symptom over-reporting. These studies are especially informative because they converge on a common point: face-valid self-report measures and simple pre-test deterrence strategies appear insufficient when symptom exaggeration is intentional and contextually motivated.

The largest remaining group of studies yielded inconclusive, mixed, or context-dependent findings. Fuermaier et al. (2018), Ord et al. (2021a), and Harrison et al. (2022) found that some indices or combined scores were informative, but not accurate enough to function as stand-alone detectors across settings. Hoelzle et al. (2019), Hirsch and Christiansen (2018), and Shura et al. (2017) further showed that validity-test failure or invalid responding can complicate interpretation of symptom and neuropsychological profiles, while also demonstrating that noncredible performance does not map neatly onto ADHD diagnosis itself. Other studies in this group were relevant mainly because they highlighted vulnerability to distortion, heterogeneity of presentation, or limitations in generalizability rather than validating a specific detection method, including Harrison et al. (2007), Booksh et al. (2010), and Ramachandran et al. (2019, 2020). This point is particularly relevant from a psychiatric research perspective, because several of the most encouraging findings were generated in analogue studies with instructed simulators, whereas real-world referral samples were more heterogeneous, more comorbid, and less cleanly separable. In this sense, the gap between analogue detection and clinical applicability remains one of the central unresolved issues in the field.

Taken together, these findings suggest that the major contribution of the literature is not the identification of a single best test, but the progressive clarification of a more defensible diagnostic strategy. Detection appears more credible when adult ADHD assessment integrates developmental corroboration, collateral history, symptom scales, stand-alone and embedded validity measures, and cautious interpretation of neuropsychological data only after response validity has been addressed (Williamson et al., 2014; Marshall et al., 2016; Harrison et al., 2022; Suhr et al., 2008). Therefore, the main novel insight emerging from this review is that malingering in adult ADHD should be conceptualized less as a problem solvable by one instrument and more as a problem of diagnostic architecture. The more the assessment relies on isolated self-report and poorly corroborated symptom narratives, the more vulnerable it becomes to distortion. By contrast, the more the evaluation is structured, longitudinally informed, and methodologically plural, the greater the likelihood of distinguishing genuine ADHD from noncredible presentations in a clinically meaningful way.

4. Discussion

4.1. Clinical features suggestive of genuine ADHD versus malingering

Individuals with genuine ADHD generally show a more coherent pattern of impairment across tasks and areas of daily life, consistent with their developmental and clinical history (American Psychiatric Association, 2022; Faraone et al., 2024). By contrast, individuals simulating ADHD may report selective or exaggerated deficits while preserving broader overall functioning and may present in contexts in which obtaining a diagnosis rather than clarifying longstanding difficulties, appears to be the primary objective (Harrison et al., 2007; Marshall et al., 2016; Sadek, 2022). Although such features are not sufficient on their own, they may indicate the need for a more in-depth assessment.

Personality disorders, mood disorders, and affective temperaments should also be carefully evaluated (Asherson et al., 2014). Some patients may identify more readily with a less stigmatized condition such as ADHD than with a mood or personality disorder, which makes differential diagnosis a central task for the specialist (Harrison and Armstrong, 2016; Lovett and Harrison, 2021). The clinician must therefore distinguish ADHD from alternative explanations and, when necessary, provide appropriate feedback to the patient. At the same time, this process must be conducted carefully so as not to alienate or stigmatize individuals who may have genuine difficulties, thereby preserving the therapeutic relationship and trust in the healthcare system.

4.2. Practical recommendations for assessment

When malingering is possible, the assessment should rely on structured clinical judgment and multiple independent sources of information rather than on any single symptom scale, SVT, or PVT. General strategies include:

1. the integration of ADHD symptom scales, collateral developmental information, behavioral observations, cognitive data, SVTs, and PVTs, which may improve interpretive confidence (Nelson and Lovett, 2019);
2. the interpretation of validity findings in relation to individual history, symptom course, comorbidities, external incentives, and demographic context, which may help adapt the diagnostic process to the patient's clinical profile (Lovett and Harrison, 2021);
3. longitudinal follow-up, when feasible, because observing symptom stability and functional impairment over time may help distinguish persistent ADHD from noncredible or situational presentations (Faraone et al., 2024; Roor et al., 2024).

Given the challenges highlighted in this review, standard ADHD assessments should be supplemented with explicit validity assessment, including SVTs, PVTs, and embedded validity indicators when clinically appropriate (Nelson and Lovett, 2019; Sweet et al., 2021; Ovsiew et al., 2023). Performance validity tests such as the TOMM, WMT, LMT, DMT, NV-MSVT and MSVT should be considered useful adjuncts not because they were frequently used, but because several studies found that stand-alone PVTs and multi-test validity approaches showed high specificity and moderate or variable sensitivity in detecting feigned or noncredible ADHD presentations (Suhr et al., 2008; Sollman et al., 2010; Jasinski et al., 2011; Williamson et al., 2014; Spenceley et al., 2022).

The strongest quantitative example was Jasinski et al. (2011), where failure of two or more validity tests produced specificity of 1.00 but sensitivity of 0.475. This means that multiple validity-test failures may strongly increase suspicion of noncredible performance, whereas passing validity tests does not exclude feigning or symptom exaggeration. Other studies were consistent with this interpretation: Sollman et al. (2010) reported moderate sensitivity and high specificity for TOMM, DMT, LMT, and NV-MSVT, Williamson et al. (2014) found that TOMM, LMT, and NV-MSVT differentiated malingered ADHD from ADHD with and without comorbidity, and Spenceley et al. (2022) reported that embedded WJ-IV indices detected at least 50% of simulators while maintaining specificity of at least 90%. Stand-alone PVTs are relevant because they assess performance on tasks that are generally easy even for individuals with genuine cognitive difficulties, so markedly poor performance may suggest noncredible cognitive performance rather than true ADHD, although PVT failure should not automatically be equated with malingering (Hirsch and Christiansen, 2018; Shura et al., 2017; Sweet et al., 2021).

The clinical interview must also extend beyond the patient's immediate report and explore developmental history, behavioral patterns, functional impairment, and the motivation for seeking a diagnosis (American Psychiatric Association, 2022; Nelson and Lovett, 2019; Lovett and Harrison, 2021). SVTs and self-report validity indicators also showed clinically useful but heterogeneous performance. PAI-derived ADHD-specific algorithms showed high specificity, especially at item level, whereas SAMS showed high sensitivity but lower specificity (Aita et al., 2018; Ramachandran et al., 2019). CAARS, BADDS, WHODAS 2.0, and other face-valid self-report instruments were vulnerable to inflation or insufficient as stand-alone credibility indicators (Harrison et al., 2007; Tucha et al., 2009; Bryant et al., 2018; Walls et al., 2017). Therefore, SVTs, PVTs, and embedded validity indicators should be interpreted as convergent evidence rather than as definitive classification tools.

Whenever possible, collateral information from family members, teachers, or colleagues should also be obtained (American Psychiatric Association, 2022; Nelson and Lovett, 2019). This multi-method approach is particularly important because it allows the clinician to compare self-report, observed behavior, cognitive performance, validity findings, and developmental history across sources. In this way, it becomes more difficult for coached or exaggerated presentations to remain undetected, especially when current protocols rely too heavily on self-report measures alone. Although this approach is more resource-intensive, it appears necessary when malingering is suspected. Clinically, the most defensible interpretation is therefore asymmetric: multiple failed PVTs or convergent SVT/PVT abnormalities increase concern for noncredible presentation, whereas normal validity findings do not by themselves establish that the ADHD presentation is genuine.

Most of the reviewed studies focused on psychological and behavioral assessment, whereas biometric and neurophysiological markers remain only marginally explored. Fuermaier et al. (2018) explored the utility of vigilance tasks and motion tracking in detecting malingering, although motion tracking offered only limited additional value. Outside formal testing, clinicians may also attend to features such as response latency, repetitive symptom descriptions, excessive emphasis on deficits, or discrepancies between declared difficulties and observed behavior during assessment. These indicators are not diagnostic evidence, but they may justify deeper evaluation (Sansone and Sansone, 2011; Sadek, 2022).

4.3. Unraveling the tangle: motivations underlying malingering in adult ADHD assessment

It is essential to understand the motivations that may underlie malingering in ADHD assessment (Sansone and Sansone, 2011; Sadek, 2022). Identifying these motives is complex, because they vary across individuals and contexts. However, the literature reviewed here suggests several recurrent external incentives, including academic accommodations, access to stimulant medications, and the avoidance of academic or occupational demands (Harrison et al., 2007; Marshall et al., 2016; Ramachandran et al., 2020). These motivations are particularly relevant in adult ADHD assessment because they may directly influence how symptoms are described and emphasized during the diagnostic process. A further contemporary concern is the increasing accessibility of online ADHD-related information and self-assessment material (Abdelnour et al., 2022; Karasavva et al., 2025). Social media content, commercial screening tools, and online questionnaires may encourage self-diagnosis or provide individuals with symptom descriptions that can later be reproduced during clinical evaluation. In this context, the role of the specialist is pivotal, because the clinician must distinguish between genuine ADHD symptoms, symptoms shaped by other psychiatric conditions, and presentations influenced by social pressures, expectations, or external gain (American Psychiatric Association, 2022; Lovett and Harrison, 2021). The broader social context should also be considered. In environments in which performance demands are high, an ADHD diagnosis may at times be perceived as a way to explain academic or occupational difficulties, gain access to accommodations, or obtain medication (Harrison and Rosenblum, 2010; Sadek, 2022). For this reason, misdiagnosis is clinically consequential and may lead to inappropriate treatment, unnecessary exposure to stimulant medication and its adverse effects, and inefficient allocation of clinical resources (Abdelnour et al., 2022; Faraone et al., 2024). At the same time, it may undermine the legitimacy of individuals who genuinely require support. Specialists therefore play a central role not only as diagnosticians, but also as guides in ensuring that diagnoses are evidence-based, clinically grounded, and not driven by uncorroborated self-report or external incentives, in accordance with the principle of “first, do no harm”.

CRediT authorship contribution statement

Giorgio Ratti: Writing – original draft, Conceptualization. **Antonio Callari:** Writing – original draft, Conceptualization. **Guido Nosari:** Writing – original draft. **Aida Peyvandi:** Writing – review & editing. **Alessandro Rodolfo Bianchi:** Writing – review & editing. **Giuseppe Delvecchio:** Writing – review & editing, Supervision.

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Declaration of competing interest

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